Asthma Management – NDIS In-Home Support Scenarios

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Five branching workplace scenarios between a disability support worker and their team leader, set in a participant's own home.

How the scenarios work

The learner plays Jess, a support worker who often works alone in a participant's home. Her team leader is usually on the phone, not in the room, so each scenario tests what Jess does on her own and when she escalates.

Character	Role	Notes
Jess Kowalski	Disability support worker, the learner's role	Two years' experience, does weekday shifts with Liam
Daniel Osei	Team leader	Supervises a team of in-home support workers, available by phone
Liam Carter	NDIS participant, 31	Lives in his own unit, has an intellectual disability and asthma, speaks in short sentences, loves gardening and the footy
Karen Carter	Liam's mum	Visits weekly, helps with GP appointments

Every lesson follows the same pattern.

1. **Situation:** a short scene between Jess, Liam and Daniel, ending on a dilemma.
2. **Decision point:** three options for what Jess does next.
3. **Consequences:** what happens after each option.
4. **Debrief:** the stronger choice and why.
5. **On the floor:** practical actions for the learner's next shift.

Scope note: support workers act within their provider's medication policy, their training and the participant's support plan. These scenarios reflect that. Nothing here replaces the participant's own asthma action plan, which always comes first. Liam's choice and control run through every lesson: Jess supports his decisions and explains risks in ways he understands.

Lesson 1 – What asthma is

Learning outcome: recognise the everyday signs that a participant's asthma is not well controlled, even when they don't say so, and report them.

Situation: "Just a cold"

[Liam's unit, Tuesday, 9:10 am. Jess arrives for a morning shift. Liam is still in his dressing gown at the kitchen table. He coughs, a dry, tight cough.]

JESS: Morning, Liam! Ready for the garden today?

LIAM: Tired. Coughing. No sleep.

JESS: Coughing all night?

LIAM: [nods] Bad night. Again.

[Later, in the back garden. Liam starts weeding, then stops after a few minutes and sits on the step, breathing fast. He says nothing. Jess notices his blue reliever puffer on the bench, and the counter shows it's nearly empty. It was new two weeks ago.]

[Jess steps inside and calls Daniel.]

JESS: Hi Daniel. Liam's fine, just a cold I think. He's coughing a bit and tired.

DANIEL: Coughing at night?

JESS: He said a bad night, again. And he sat down in the garden after five minutes. That's not him. He usually does an hour.

DANIEL: And his puffer?

JESS: Nearly empty. It was new a fortnight ago.

DANIEL: Okay. Night cough, stopping activity he enjoys, and using his reliever a lot. That doesn't sound like just a cold to me. What do you think we should do?

Decision point

- **Option A:** Note "Liam has a cold" in the shift notes and let the next worker keep an eye on it.
- **Option B:** Record exactly what she's seen, with times, tell Daniel it's a pattern, and agree a plan to get Liam's asthma reviewed by his GP.
- **Option C:** Give Liam his reliever every few hours for the rest of the shift, just to be safe.

Consequences

A: [Saturday, 2 am. An on-call message lands on Daniel's phone from the overnight worker: Liam has been taken to hospital by ambulance.] **DANIEL:** [Monday, to Jess] Three bad nights in a row were in the notes as "a cold". If we'd joined the dots on Tuesday, his GP could have seen him before it got this far.

B: [Thursday afternoon. Jess and Karen with Liam at the GP clinic.] **DANIEL:** [on the phone afterwards] How did it go? **JESS:** The GP said his asthma's not well controlled. She's changed his preventer and wants to see him in four weeks. Liam liked that I wrote down the garden thing, he said "yes, that".

C: DANIEL: Giving extra reliever on a schedule isn't in his plan, and it hides the problem. His puffer would run out faster, and nobody would know his asthma was getting worse.

Debrief

Stronger choice: Option B. Asthma is a long-term condition where the airways become swollen and narrow. When it isn't well controlled, the signs are often small and spread across days.

- **Option A** treats a pattern as a one-off. Small signs that aren't reported can build into an emergency.
- **Option C** is well-meant, but it's outside Liam's plan and outside Jess's role. It masks the warning signs.
- **Option B** describes facts, not guesses, so the team leader, family and GP can act early.

Common signs of poorly controlled asthma include coughing or waking at night, needing the reliever more often than usual, and avoiding activities that used to be easy. Many participants won't say "I'm short of breath". They may stop, sit down, seem irritable or tired, or avoid stairs.

On the floor

- Learn what's normal for each participant, so you notice when it changes.
- Write what you see and hear, with times: "sat down after 5 minutes of weeding, breathing fast", not "has a cold".
- Check the reliever's dose counter when you check other medications.
- Report a pattern to your team leader on the same shift.
- Ask the participant in simple words: "Is breathing hard?" and watch for nods, gestures or behaviour.

Lesson 2 – Triggers

Learning outcome: identify a participant's asthma triggers, and reduce exposure while respecting their choice and control.

Situation: The garden centre trip

[Liam's lounge room, a warm, windy Thursday in November, 10 am. Liam has his shoes on and his wallet ready. Jess checks her phone: a thunderstorm asthma warning is out for the afternoon, with high grass pollen and storms due around 3 pm.]

LIAM: Garden centre. Today. Tomatoes.

JESS: You've been waiting all week for this, haven't you?

LIAM: [grins] Big tomatoes.

[Jess steps into the kitchen and calls Daniel.]

JESS: Daniel, there's a thunderstorm asthma warning today. Liam's got hay fever and asthma. He's desperate to get his tomato plants. I don't want to take that off him.

DANIEL: Good that you checked the warning. What's in his support plan about triggers?

JESS: Pollen, smoke and cold air. And it says he likes things explained with pictures.

DANIEL: So the risk is real, and so is his choice. It's his life and his decision. Our job is to help him make it with the right information. What are the options?

JESS: We could go now, before the storm. Or I could ring them and see if they deliver.

DANIEL: Either could work. Make sure his puffer and spacer go with you, whatever he picks.

Decision point

- **Option A:** Tell Liam the trip is cancelled because of the weather, and suggest a movie instead.
- **Option B:** Explain the warning to Liam simply, offer him choices (go now and be home by 1 pm, or order for delivery), pack his reliever and spacer, and plan to close the windows before the storm.
- **Option C:** Go to the garden centre as planned. It's a short trip, so the puffer can stay at home.

Consequences

A: [Afternoon. Liam sits with his arms folded, refusing lunch.] **LIAM:** My tomatoes. My money. **DANIEL:** [on the phone later] He was safe, but he had no say. Next time that kind of decision can feel like being told off, and he may stop telling us his plans.

B: [Back home at 12:30 pm, tomato seedlings on the bench. At 3 pm, dark clouds roll in. Liam and Jess close the windows together.] **LIAM:** Storm. Windows shut. **DANIEL:** [on the phone] Nicely done. He chose to go early, he got his tomatoes, and he was inside before the storm. Pop the warning and his choice in the notes.

C: [Garden centre car park, 1:45 pm. The wind picks up, grass pollen in the air. Liam starts coughing and wheezing.] **JESS:** [to herself, searching her bag] His puffer's on the bench at home. **DANIEL:** [later] The trip itself wasn't the mistake. Leaving his reliever behind on a warning day was.

Debrief

Stronger choice: Option B. Reducing exposure to triggers helps prevent flare-ups, but the participant still makes the decisions about their own life.

- **Option A** keeps Liam safe today, but takes away his choice without explaining why.
- **Option C** respects his choice, but ignores a known high-risk trigger and leaves him without his reliever.
- **Option B** shares the risk in a way Liam understands, gives him real options, and plans for safety whichever he picks.

Common asthma triggers include colds and flu, pollen, smoke (cigarette, bushfire and wood fire), cold air, dust, mould, strong smells such as sprays and perfume, exercise and stress. Each participant's triggers are different, so check their support plan and ask them

and their family. In Victoria, epidemic thunderstorm asthma can happen in grass pollen season, usually October to December.

On the floor

- Know each participant's triggers, as listed in their support plan.
- In spring, check the thunderstorm asthma and pollen forecast at the start of your shift.
- Explain risks with plain words, pictures or the participant's preferred communication style.
- Offer real choices, and record what the participant decided and why.
- Make the reliever and spacer go everywhere the participant goes.

Lesson 3 – Medications and devices

Learning outcome: tell a preventer from a reliever, spot device problems, and support good technique within your role.

Situation: The new preventer

[Liam's kitchen, Monday, 8:30 am, three weeks after the GP visit. Daniel has come for a supervision visit. Liam takes his new preventer puffer from the cupboard, puts it straight in his mouth, presses it and breathes in fast. He puts it away and picks up his toast.]

DANIEL: [quietly, to Jess] Is that how he usually takes it?

JESS: Every morning. He likes to do it himself.

DANIEL: Good, he should. Where's his spacer?

JESS: [opens a drawer] Here. I washed it on Friday and dried it with a tea towel so it was ready.

DANIEL: Tea towels leave static inside, so less medicine reaches his lungs. Spacers should drip-dry. Did the GP say to use the spacer with the preventer?

JESS: I think so. It's on the medication chart. And she said rinse his mouth after.

DANIEL: [checking the blue reliever] This one expired in August.

JESS: Oh. I didn't think to check dates on the puffers.

LIAM: [looking up] My puffer. I do it.

DANIEL: You do, Liam. We just want to make sure it's working for you. Jess, what do you reckon?

Decision point

- **Option A:** Take over giving Liam's preventer every morning, so it's done properly.
- **Option B:** Support Liam to use the spacer as the chart says, prompt the mouth rinse, replace the expired reliever, and ask for a pharmacist to check his technique and provide a picture guide.
- **Option C:** Leave it. Liam has used puffers for years and knows his own routine.

Consequences

A: [Two weeks later.] **JESS:** [on the phone] Liam won't let me near his puffer now. He's hiding it in his room. **DANIEL:** He's lost something he was proud of doing himself. We need to rebuild that.

B: [Pharmacy, the next week. The pharmacist shows Liam how to use the spacer and gives him a laminated card with pictures. At home, it's stuck on the fridge.] **LIAM:** [pointing at the card] Spacer. Four breaths. Water. **DANIEL:** [on the phone] Same independence, better technique. And the new reliever's in date? **JESS:** In date, and I've added puffer expiry dates to my weekly medication check.

C: DANIEL: Without the spacer, most of that preventer is ending up in his mouth and throat, not his lungs. And if he'd needed his reliever, it was out of date.

Debrief

Stronger choice: Option B. A medicine only helps if it reaches the lungs. Good support keeps the participant in charge and makes their routine work.

- **Option A** gets the dose in, but removes Liam's independence without a reason in his plan.
- **Option C** respects his routine, but ignores the chart, an expired reliever and a technique problem.
- **Option B** fixes the problems, follows the medication chart and builds Liam's own skills.

Item	What it does	What to check
Preventer	Reduces swelling in the airways over time. Taken every day, even when well.	Taken as prescribed, mouth rinsed after if the chart says so
Reliever (usually blue or grey)	Opens the airways within minutes. Used for symptoms and first aid.	In date, dose counter not near zero, always with the participant
Spacer	Helps more medicine reach the lungs	Clean, drip-dried, not cracked, goes everywhere with the reliever

On the floor

- Follow the participant's medication chart and your provider's medication policy.
- Only support medication tasks you've been trained and authorised to do.
- Check expiry dates and dose counters on all puffers each week.
- Wash spacers in warm soapy water and let them drip-dry.
- Ask for a pharmacist or GP technique check if you notice a problem.

Lesson 4 – Action plans

Learning outcome: read a participant's written asthma action plan, match their symptoms to the right section and act on it.

Situation: Getting worse

[Liam's unit, Wednesday, 11 am. Liam is on the couch with a box of tissues. He has a cold. His asthma action plan from the GP is stuck on the fridge, beside the pharmacist's picture card. Jess has the shift notes open on her phone.]

JESS: [on the phone] Daniel, Liam's got a cold. The overnight notes say he used his reliever twice last night, and he's used it once this morning.

DANIEL: Is he managing to talk and move around okay?

JESS: Talking fine. A bit wheezy. He's watching the cricket.

DANIEL: Have you looked at his action plan?

JESS: [walking to the fridge] It's got three parts. "When well", "When getting worse" and "Emergency".

DANIEL: Which part matches how he is right now?

JESS: Getting worse, I think. It says if he needs his reliever more than usual or wakes at night with asthma, keep taking the preventer and see the GP within a day or two.

DANIEL: Karen's taking him out Saturday. She might say wait till then.

LIAM: [from the couch] No doctor. Cricket.

JESS: [smiles] We'll work around the cricket, mate.

Decision point

- **Option A:** Wait for Karen's visit on Saturday and see how Liam is then.
- **Option B:** Follow the "getting worse" section: keep his preventer going, book a GP appointment within the timeframe on the plan (working around the cricket), let Karen know, and record his reliever use.
- **Option C:** Give Liam an extra dose of his preventer tonight to get ahead of it.

Consequences

A: [Friday night. A message from the overnight worker to Daniel: Liam is wheezing and struggling to speak in full sentences. They've started asthma first aid.] **DANIEL:** [the next day] The plan told us what to do on Wednesday. We waited for Saturday instead.

B: [Thursday morning. Liam, Jess and Karen at the GP. The GP adjusts his treatment for a few days and updates his plan.] **KAREN:** [to Jess] Thanks for not waiting for me. **DANIEL:** [on the phone] Now everyone reading the plan knows exactly what to do next time. Good work.

C: DANIEL: Changing a dose is a prescriber's decision, not ours. If the plan doesn't tell you to change it, don't. Tell me, and we'll get him seen.

Debrief

Stronger choice: Option B. A written asthma action plan is written by the participant's doctor for exactly this moment. It tells everyone what to do, so nobody has to guess.

- **Option A** puts a family visit ahead of the plan's timeframe, and the asthma kept getting worse.

- **Option C** feels proactive, but it changes a prescribed dose outside the plan and outside Jess's role.
- **Option B** matches the symptoms to the right section, follows it, and keeps Liam, his family and the team informed.

Most action plans have a section for when asthma is well controlled, one for when it's getting worse, and one for an emergency. Each participant's plan is different, so read theirs, not a general one. Ask for it to be reviewed if it's out of date or hard to follow.

On the floor

- Know where each participant's asthma action plan is kept, and read it at the start of your first shift with them.
- Match what you see to a section of the plan, and do what that section says.
- Never change a dose unless the plan or prescriber tells you to.
- Tell your team leader and the participant's supporters when the "getting worse" section applies.
- Ask for an accessible version (pictures, easy read) if the participant would benefit.

Lesson 5 – Asthma attack first aid

Learning outcome: recognise a severe asthma attack, call 000 and give asthma first aid while working alone, then report the incident.

Situation: Alone in the garden

[Liam's back garden, a windy Friday, 2:15 pm. Jess is inside writing shift notes. The back door bangs open. Liam comes in, hunched forward, gripping the bench. His breathing is loud and fast, and his shoulders rise with each breath.]

JESS: Liam? What's happening?

LIAM: Can't... breathe.

JESS: Okay. Can you tell me more?

LIAM: [shakes his head, pointing at his chest] Tight.

[He can only manage one or two words at a time. His lips look pale. Jess is the only worker on shift. Her phone is in her hand. Liam's reliever and spacer are in the kitchen drawer, beside his action plan on the fridge.]

JESS: [to herself] Daniel. Should I ring Daniel?

Decision point

- **Option A:** Call Daniel first and ask him what to do.
- **Option B:** Sit Liam upright and stay calm with him, call 000 on speaker, and start asthma first aid with his reliever and spacer. Call Daniel once the ambulance is on its way.
- **Option C:** Help Liam lie down on the couch to rest, and give him several quick puffs of his reliever straight from the puffer.

Consequences

A: [Daniel's phone rings out. It goes to voicemail. Two minutes pass.] **JESS:** [leaving a message, voice shaking] Daniel, it's Liam, he can't breathe... **DANIEL:** [later] I'm always glad you call, but not first. In an emergency, 000 and first aid come before anything else, including me.

B: [Jess sits Liam on a kitchen chair, leaning slightly forward. Phone on speaker on the bench.] **JESS:** Ambulance. My client has asthma and can only say one or two words. [She shakes the reliever, attaches the spacer.] **JESS:** One puff, Liam. Now four breaths. Good. [Four minutes later, still wheezing, she gives four more.] [Paramedics arrive at 2:29 pm. Liam is breathing a little easier.] **DANIEL:** [on the phone afterwards] You did exactly the right things, in the right order. Fill in the incident report while it's fresh, and how are *you* going?

C: [Liam lies on the couch, breathing harder. Jess presses the puffer three times in a row.] **LIAM:** [gasping, trying to sit up] No... **JESS:** [realising, helping him sit up] Okay, okay, sit up. [Later, Daniel explains:] Lying down made it harder for him to breathe, and quick puffs without the spacer meant very little medicine reached his lungs.

Debrief

Stronger choice: Option B. Liam could only speak a word or two, which is a sign of a severe attack. That means calling 000 straight away and starting first aid without delay.

- **Option A** delays care while Jess waits for advice she doesn't need to wait for.
- **Option C** is a natural instinct, but lying down makes breathing harder, and puffs without the spacer deliver less medicine.
- **Option B** follows the steps of asthma first aid and gets help on the way early.

Asthma first aid with a blue or grey reliever puffer:

1. Sit the person upright, stay with them and be calm and reassuring.
2. Give 4 separate puffs of the reliever through a spacer: 1 puff at a time, with 4 breaths after each puff.
3. Wait 4 minutes. If they still can't breathe normally, give 4 more puffs.
4. If they still can't breathe normally, call 000. Keep giving 4 puffs every 4 minutes until the ambulance arrives.

Call 000 straight away if the person is not breathing, gets suddenly worse, has severe signs (such as only speaking a few words, or blue or pale lips), has no reliever, or you're not sure it's asthma. If the participant's action plan or reliever uses different first aid steps, follow those.

On the floor

- Know where the reliever, spacer and action plan are at the start of every shift.
- In an emergency, call 000 before calling your team leader.
- Use your phone's speaker, so you can call and give first aid at the same time.
- After the emergency, tell your team leader and complete an incident report through your provider's system. Your team leader will decide whether it needs to be reported to the NDIS Quality and Safeguards Commission.
- Look after yourself afterwards. Talk to your team leader or your employee support line.

Sources

Check first aid steps against the current charts before publishing, as guidance is updated from time to time.

- Asthma emergency first aid, Better Health Channel (Victorian Government)
- Australians urged to know asthma first aid, National Asthma Council Australia
- Asthma emergency: how to use new formulations, The Medical Republic